

إسم المريض رابعياً : حمدية سعد السيد بدر الدين

المحافظة : الجيزة الحي / المدينة : الهرم

العنوان : --

تاريخ تسجيل الإستمارة : 26-09-2026

رقم البطاقة : 26507111700601 رقم التلفون : 01152606026

العمر(تاريخ الميلاد) : 11-07-1965 المهنة : ربه منزل

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-
الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة
نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي (-----)
المقرر بما فيه التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطه العلاج : علاج مبادرة سرطان الثدي علاج كيماوي تاكسان (دوسيتاكل) [14153]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد کیلانی

الاسم: د/ندی حسن

الاسم: د/نرمين رمضان

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

الوظيفة: ----- التوقيع:

تتعمد صحة البيانات المرفقة
رئيس اللجنة و مدير
المستشفى.

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقر اللجنة بأن البيانات المدونة في هذا النموذج

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

----- / د ----- / د ----- / د. | ----- / د. |

قرار اللجنة

التشخيص النهائي :

خطة العلاج :

المستشفى :

جهة العلاج : --

ملاحظات : ---

ختم

شعار الجمهورية

مدير عام

مديرية الشؤون الصحية

رئیس

المجلس الطبي المتخصص

--

Name: حميدة سعد السيد بدر الدين National ID: 26507111700601	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Docetaxel chemotherapy

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



7.7.1.1 150.41114.1.1



أنشئ مسجلة متزوجة
عبد الرحمن بن عبد العزى بن
البطاقة سارية حتى ٢٠٢٧/٠١/٢٠



جمهورية مصر العربية
بطالة تحلق الشخصية

حماد

سعد السيد بدر الدين

٦ ش. القدم الشريف - المساحه

الهرم - الجيزة

1960-55

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Patient Name : الأستاذة / جندية محمد السيد	Request No. : 13007054
Patient No. : 26507111700601	Request Date : 06/01/2024
No. : 59/24	Verification Date : 20/01/2024
: Female	Financial Category : نفقة الدولة - اورام دار السلام
: 58 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

clinical Data :

History: breast cancer post NACT.

Specimen: core of the specimen :

Specimen: MRM specimen collected at 6/1/2024.

Examination:

Specimen: MRM specimen measured 24x20x4 cm , with covering skin ellipse measured 19x15 cm

Specimen: gross extent : An ill defined soft to rubbery greyish white tumour bed site

Specimen: size: 5x4x3 cm .

Specimen: Nipple & areola were unremarkable grossly

Specimen: distance from margins: 1 cm away from deep margin

Specimen: lymph nodes number and size: 9 nodes up to 1.5x1 cm .

Specimen: additional pathological findings: A well defined gray white rubbery nodule measured 1.5 cm.

Specimen: microscopic examination:

Specimen: No evidence of tumor regression in a previously diagnosed carcinoma.

Specimen: No evidence of invasion: Not detected.

Specimen: Lymph node involvement: Free (0/9)

Specimen: Nipple involvement : Free nipple and areola.

Specimen: Surgical Margins: Free deep margin .

Specimen: Histologic response to therapy: Pathologic complete response .

Specimen: Additional pathological findings: Fibroadenoma with evidence of hemorrhage and myxoid changes.

Specimen: Lesions:

Specimen: Diagnosis:

Specimen: Breast cancer post NACT , MRM specimen :

Specimen: Pathologic complete response in a previously diagnosed invasive carcinoma.

Specimen: Nipple and areola.

Specimen: Deep margin.

Specimen: Fibroadenoma with evidence of hemorrhage and myxoid changes.

Verified by :

Dr. esraa magdy

Name: <u>حمدة سعد السيد بدر</u>		
MR. : <u>2235</u>		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالتها		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انها		
The patient was diagnosed with triple-positive breast cancer (TPBC) and underwent right modified radical mastectomy (MRM) in 2024. Pathology revealed residual disease, and adjuvant therapy with PHESGO, placement of a port, and ARAMIDEX was initiated in May 2025. In February 2026, metastatic disease to the skull bone was identified at multidisciplinary tumor board (MDT). A computed tomography of the chest, abdomen, and pelvis is pending, and palliative radiotherapy to the skull bone is ongoing. Noadjuvant therapy with TCHP was administered, and the patient continued adjuvant PHESGO and aromatase inhibitor (AI) via the port through November 2025. Skull bone metastases persisted, and palliative radiotherapy was delivered. Staging CT of the chest and abdomen showed no additional metastases, while bone scan confirmed multiple skull bone lesions in June 2026. The MDT recommended weekly taxol and Herceptin. The patient prefers chemotherapy every 21 days for logistical reasons, has no diabetes mellitus or hypertension, and is scheduled to receive docetaxel plus intravenous Herceptin. Herceptin approval is pending. Weight is 67 kg, height 160 cm, and the plan is to start cycle 1 of docetaxel at 70 mg/m² after laboratory results; double dexamethasone and methylalntrexone were administered, and breast examination was normal. The patient has initiated cycle 1 of docetaxel. Clinical pharmacy note indicates a surface area of 1.63 m² for cycle 1 taxotere (114 mg) and a pending approval for Herceptin loading dose. The patient has received cycle 1 of taxotere with a loading dose of Herceptin. For week 4, taxol 60 mg/m² was administered with supplemental therapy after laboratory confirmation; the patient also received cycle 1 Herceptin loading dose. Subsequent taxol doses were given in weeks 5 and 6. The patient has completed week 4 taxol, received the loading dose of Herceptin in cycle 1, and completed weeks 5 and 6 of taxol. Metastatic breast cancer to the skull was treated with radiotherapy (8 Gy/8 fractions). The patient received Herceptin 405 mg in cycle 2, taxol 135 mg/m² in week 7, and paclitaxel in weeks 8 and 9, achieving a 64 % response (19/7) with a surface area of 1.7 m². The patient has completed cycle 2 of Herceptin and has received taxol in week 7 and paclitaxel in weeks 8 and 9.		
تحريرا في ,		
	التاريخ : 20-09-2026	الوقت :
	اسماء محمد مصطفى نصر	الطبيب المعالج :
يعتمد ,		

Presidential Initiative for Women's Health - Pathology Lab

المبادرة الرئاسية لصحة المرأة - معمل الباثولوجي

Name : الأستاذة / حدييه سعد السيد	Request No. : 13007054
No. : 26507111700601	Request Date : 06/01/2024
Age : 59/24	Verification Date : 20/01/2024
Sex : Female	Financial Category : نفقة الدولة - اورام دار السلام
Age : 58 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

lymph nodes (0/9).
ypT0 , ypN0.

Samal Abdelatty
Magda Elkady

Verified by :
dr.esraa magdy

g Date : 27/04/2024 09:24:28

Page 2 of 2

Printing By : marwa mohamed youss

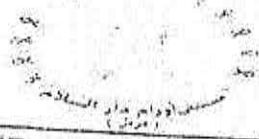


Dar El-Salam Cancer Center

Radiology Department

مركز أورام دار السلام

قسم الأشعة



Name: HAMDEYA SAD AL-SAYED

Date : 22 February 2025

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- average in size , smooth outline, homogenous echopattern. No intra hepatic biliary radicle dilatation.
- Portal vein is not dilated and patent.

Gall Bladder: -

- Average in size and in wall thickness , no mud or stones could be seen.
- CBD is not dilated.

Spleen: -

- Average size , displaying homogenous echopattern.

Kidneys: -

- Are of normal size and site , normal echogenicity . No sizable stones backpressure changes , no solid or cystic lesion.
- Para-aortic and pancreatic area: no gross lesion
- No ascites could be seen.

UTERINE BLADDER: empty at scan time.

UTERUS: average in size and in endometrial thickness, no focal lesions could be seen.

adnexal masses or cysts ,
ascites .

Opinion:

d colonic gaseous distension , otherwise unremarkable abdominal and pelvic ultrasound .

Best Regards,
Dr. A. Haider



Patient Name: Hamdoya Saad Al Sayed Badr Al Din	Date : 07/10/2024
Exam : MRI LUMBER SPINE	Age : 59 Year(s)
Medical ID: 2229/2023	

MRI EXAMINATION OF THE LUMBAR SPINE

Technique Of Examination: -

- Sagittal T1 & T2 WIs
- Axial T2 WIs

MRI Findings:

- Preserved lumbar lordotic curve.
- Normal dimensions of the bony lumbar spinal canal.
- Lumbar spondylodegenerative changes manifested by variable degrees of reduced T2 signal brightness of the lumbar discs with anterior marginal osteophytes, & Modic-II lower VEPs degeneration.
- L5-S1 leftward inclined posterior disc bulge is seen indenting the thecal sac ventrally and encroaching on the exit neural foramina more on the left side.
- L4-5 diffuse posterior disc bulge is seen indenting the thecal sac ventrally and encroaching on the exit neural foramina.
- L3-4 biposterolateral small focal disc protrusions noted.
- Normal appearance of the conus medullaris and cauda equina nerve roots.
- Mild lower facet arthropathy.
- Normal marrow signal, no evidence of infiltrative lesions.
- No paraspinal lesions.

Opinion:

- Lumbar spondylodegenerative changes.
- Multilevel disc lesions as described, for their significance to be clinically correlated.

Dr. Sherif Sameh, M.D.

10-10-2024
10-10-2024
10-10-2024

Patient Name: Hamdeya Saad Al Sayed Badr Al Din	Date : 15/02/2025
Exam : X-RAY CHEST	Age : 59 Year(s)
Medical ID: 2229/2023	Pat. ID: 44713

Plain X-ray **Chest** (PA view) revealed:

- Clear both lung fields.
- Clear both costo-phrenic angles.
- Normal cardiac size & configuration.
- Normal thoracic cage

Signature Dr . Nermien Ramadan

24/02/2025 11:23 AM



Patient Name: Hamdöya Saad Al Sayed Badr Al Din	Date : 22/02/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 59 Year(s)
Medical ID: 2229/2023	Pat. ID: 44713

LEFT BREAST MAMMOGRAPHY WITH BILATERAL US

Technique

- LEFT breast MLO and CC views
- Bilateral ultrasound

Report:

- The left breast is scattered fibroglandular densities of the right breast (ACR B)
- No spiculated masses or grouped microcalcifications.
- Complementary ultrasound also showed no left breast solid or cystic masses
- Normal skin thickness and nipple-areola complex, with average caliber of the retro-areolar ducts
- Nonspecific left axillary LN, with preserved fatty hila and uniform cortical thickness.
- Free right operative bed with no evidence of recurrent mass lesions
- Free right supra and infra regions.

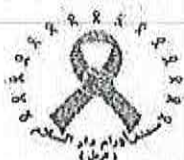
Opinion:

- Right MRM with free operative bed with no evidence of recurrent mass lesions
- No left breast suspicious findings (BIRADS 2)

Yours truly

Signature Dr. Youmna tarek

22/02/2025 1:41 PM



Patient Name: Hamdeya Saad Al Sayed Badr Al Din	Date: 06/12/2023
Exam: MAMMOGRAPHY BOTH	Age: 58 Year(s)
Medical ID: 2229/2023	Pat. ID: 44713

**BILATERAL BREAST MAMMOGRAPHY AND HIGH RESOLUTION
ULTRASOUND**

Indication:

Known case of right breast cancer on therapy with clip seen within.

Technique

- Bilateral MLO and CC views , Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Right UOQ irregular dense mass lesion with architectural distortion with underlying suspicious pleomorphic macrocalcifications. Measuring about 75x52mm (in MLO view).

Complementary ultrasound :

- Right 12 O'clock par areolar still noted parenchymal distortion with solid mass lesion beside with microcalcifications with subtle internal vascularity seen within, measuring about 27x8mm (compared to previous study about 40 x30mm.
- Another ill defined hypoechoic solid mass lesion seen at 4o'clock measuring about 7x7.5mm.
- Multiple bilateral simple cysts.
- No left solid mass lesion.
- Normal left skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
- Right no pathological axillary lymph nodes.
- Nonspecific left axillary LN.

Opinion:

- Right UOQ pathologically proven carcinoma showing partial radiological response (BIRADS 6)
- Good response of right axillary LNs
- Left breast benign finding (BIRADS2).

Signature Dr. Nehal Hamdy

06/12/2023 11:36 AM



MDT form
1. Medical number:
Female

83147918
Old Med

2229-2023

A Patient ID
1. Name:
2. ID number: 26507111700001

Patient Age 39 Y

REASON OF MDI

Date of MDI 13/02/2024

Family history: ER 01144098360
Marital status: N/A
Comorbidity: ANTIHYPERTENSIVE
Menopausal status: N/A
Initial stage: N/A

Clinical data

Breast Imaging:		Meta Workup: BONE SCAN-->FREE CTCAP-->FREE	
Bx IDC	ER 8/8	PR 5/8	Her 2 Neu 3
SISH		SISH COMMENT	
Xig7 40			
T 2	N 1	M 0	

Sonomammography
RT BREAST MASS 4X3+INTRADUCTAL EXTENSIVE+ LEVEL I PATHOLOGICAL LNS

PATIENT HISTORY
last MDT NACT (Taxol/carbo/Herceptin/Perjeta) x 18 wks. with regular clinical assessment
then redo sonomammography & consider RT WLE & SLNB according to response
20/12/2023: Sonomammo: Partial response.
RT MRM: pCR
: free margins
LN 0/9

MDT OUTCOME & PLAN

1 year phesgo (total) PORT AI adjuvant 5 years

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY Dr. H. H.	RADIOLOGY	RADIOLOGY 13/02/2024 Dr. H. H.
SURGERY	SLAGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

Name : Hamdiah Saad El-Sayed Badr al-Din

Date : 18-05-2026 08:22

Branch : Dokki

Medical No. : 377414

Ref.Physician Prof : Goustav Roussey Egypt

تعاقب مستشفيات : Ins.Company

Tc-99m MDP Whole-Body Skeletal Scintigraphy

Clinical History

- A 61-year-old female patient with a known history of breast cancer (post-mastectomy, hormonal therapy). History of two right parietal calvarial metastases treated with radiotherapy (completed 20/04/2026).

- **Indication:** Therapy response follow-up.

- **Comparison:** Prior bone scan (29/12/2025) and brain MRI (Aug/Nov 2025).

Technique

- Following the intravenous injection of Tc-99m MDP, anterior and posterior whole-body planar scans were acquired approximately 2 hours post-injection.

Findings

- Skull: Faint, non-focal right parietal uptake. Given recent radiotherapy, this likely reflects post-radiation changes rather than active residual disease. No new focal skull lesions.
- Axial skeleton: Normal symmetric tracer distribution. Mild diffuse lower lumbar uptake attributed to benign degenerative changes.
- Appendicular skeleton: Symmetric distribution. Mild non-specific uptake at shoulders and knees (degenerative joint disease). No new suspicious distant lesions.
- Soft tissue: Normal renal clearance and bladder activity. No abnormal extraosseous uptake.

Impression

- Right parietal skull uptake: Mild, non-focal, likely reflects post-radiation effect/remodeling rather than residual metastatic deposits.
- No new osseous metastases or distant disease progression; overall stable skeletal appearance.

Much oblige

Dr. Alaa Nagi, MD

Nuclear medicine

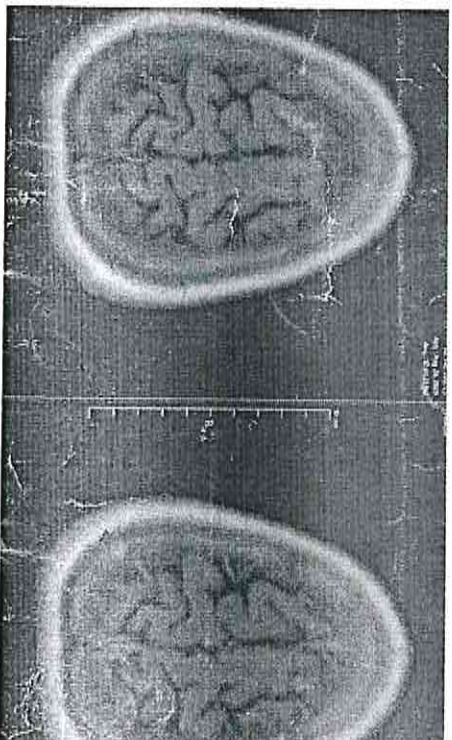
Patient Name: Hamdeya Saad Al Sayed Badr Al Din	Date : 28/08/2025
Exam : MRI BRAIN WITH CONTRAST	Age : 60 Year(s)
Medical ID: 2229/2023	Pat. ID: 44713

MRI OF THE BRAIN

TECHNIQUE:

- Sagittal T2WI
- Axial T1, T2 and FLAIR WI
- Coronal T1WI
- Post contrast T1 WIs

FINDINGS:



They

- are seen measuring 5 mm in diameter.
- Normal size and parenchymal signal intensity pattern of the cerebral hemispheres, cerebellum and brain stem.
- No intra or extra axial S.O.Ls.
- No areas of pathological enhancement.
- Normal size and configuration of the ventricular system.
- No shift of the midline structures.
- No extra-axial collections.
- Intact cranio-cervical junction.
- Partial empty sella.
- Bilateral maxillary sinusitis.

OPINION:

- Two parietal calvarial osseous lesions? metastatic, for bone scan correlation.
- Normal rest of the contrast enhanced MRI of the brain.

Dr.Sara Mahmoud, MD

Name : Hamdiah Saad El-Sayed Badr Al-Din
Branch : Dokki
Ref.Physician Prof : Goustav Roussey Egypt

Date : 29-12-2025 11:24
Medical No. : 284323
نسقة مسشغفياك : Ins.Company

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History

- A 60-year-old lady, with history of breast cancer and referred for metastatic workup.

Technique

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings

- Mild tracer uptake noted at the right parietal region of the skull.
- No other abnormal active or cold focal bony lesions that could account for metastatic bone disease.
- Degenerative changes are noted at the spine and big joints.

Impression:

- Mild tracer uptake at the right parietal region of the skull; matching with the prior MRI reported lesions therein;
- The rest of skeleton is free from metastases.

Best regards,
Dr. Alaa Nagi, MID
Nuclear Medicine

Name : Hamdeya Saad Al Sayed Badr Al Din

Date : 29-11-2025 12:04

Branch : Portable

Medical No. : GRI-44713

Ref.Physician Prof :

Ins.Company :

MRI EXAMINATION OF THE BRAIN

Technique:

- Axial T1, T2, FLAIR
- Coronal T2
- Sagittal T1
- Axial DWI and ADC

Findings:

- *Known case of metastatic breast cancer, compared to the last MRI examination dated 28/8/2025, the current study revealed:*
- No significant time interval changes regarding size and MRI signal of the previously noted two right parietal calvarial osseous lesions of low signal in T1 and bright signal in T2, the largest measures about 5 mm.
- No newly developed lesions.
- No cerebral, cerebellar or brain stem areas of altered MR signal.
- Widened cortical sulci, basal cisterns, cerebellar folia and extra-axial CSF spaces. Mild dilatation of the supra-tentorial ventricular system with no midline shift.
- No midline shift or deformity.
- No extra axial collections or masses.
- No cerebello pontine angle masses.
- No gross posterior fossa abnormalities.
- Intact cervico medullary junction.
- Partial empty sella.
- Bilateral maxillary sinusitis.
- N.B: right staphyloma for ophthalmology examination

OPINION:

- Stationary course since the last MRI examination regarding the previously noted right parietal calverial osseous lesions likely metastatic and age matching involutonal brain changes

Dr.Norhan Ramzy,MD

Name : Hamdiah Saad El-Sayed Badr al-Din

Date : 18-05-2026 08:22

Branch : Dokki

Medical No. : 377414

Ref.Physician Prof : Goustav Roussey Egypt

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Clinical History

- A 61-year-old female patient with a known history of breast cancer (post-mastectomy, hormonal therapy). History of two right parietal calvarial metastases treated with radiotherapy (completed 20/04/2026).

- **Indication:** Therapy response follow-up.

- **Comparison:** Prior bone scan (29/12/2025) and brain MRI (Aug/Nov 2025).

Technique

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Findings

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Impression

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Dr. Alaa Nagi, MD

Nuclear medicine

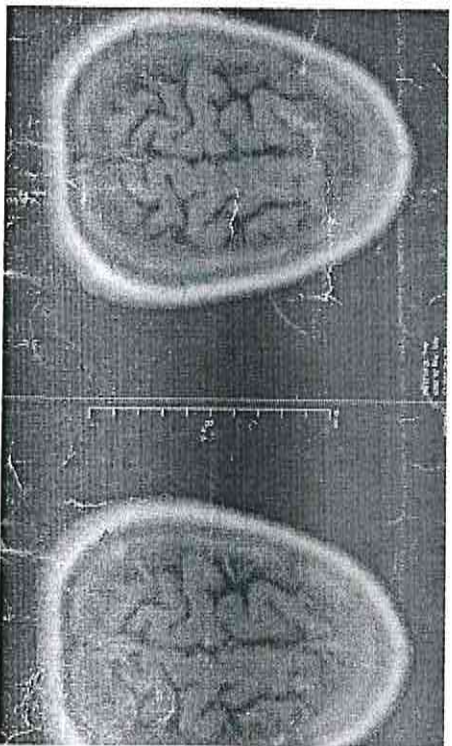
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Dr.Sara Mahmoud, MD

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Branch : Dokki
Ref. Physician Prof : Goustav Roussey Egypt

Date : 29-12-2025 11:24
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نسقة مستشفى : نسقة

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Nuclear Medicine

Name : Hamdeya Saad Al Sayed Badr Al Din

Date : 29-11-2025 12:04

Branch : Portable

Medical No. : GRI-44713

Ref.Physician Prof :

Ins.Company :

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- Coronal T2
- Sagittal T1
- Axial DWI and ADC

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- No midline shift or deformity.
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- No gross posterior fossa abnormalities.
- Intact cervico medullary junction.
- Partial empty sella.
- Bilateral maxillary sinusitis.
- N.B: right staphyloma for ophthalmology examination

OPINION:

- Stationary course since the last MRI examination regarding the previously noted right parietal calverial osseous lesions likely metastatic and age matching involutional brain changes

Dr.Norhan Ramzy,MD